Colorectal Cancer Synoptic Operative Notes

I. Basic Data & Preoperative Assessment

Field	Entry
Patient Name:	
Date of birth:	
Age:	
Sex:	
Weight:	
Heght:	
BMI:	
Operation:	☐ Colon ☐ Rectum
Operation: *(Branching logic for rectum operation)*	☐ High anterior resection ☐ Low anterior resection ☐ Intersphincteric resection ☐ Abdominoperineal resection ☐ Local excision ☐ Other (Specify)
Surgeon:	[Enter Surgeon Name]
Assistant:	1. [Enter Assistant Name] 2. [Enter Assistant Name]
Anaesthetist	[Enter name]
Duration of operation (min):	[Enter time in minutes]
ASA Score: *(Include description of classes)*	☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5

Field	Entry
Patient Name:	
Date of birth:	
Age:	
Sex:	
Weight:	
Heght:	
BMI:	
Neoadjuvant treatment: *(Branching logic for rectum operation)*	☐ Yes ☐ No
Procedure Urgency:	☐ Emergency ☐ Semi-emergency ☐ Semi-elective ☐ Elective

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Findings:

- **Description:** notes

- **Location:** *(Branching logic for rectum operation)*

  ☐ High
  ☐ middle
  ☐ low

- **Mesorectal completeness:** *(Branching logic for rectum operation)*

  ☐ complete
  ☐ Near complete
  ☐ Incomplete

- **Completeness of tumour resection:** *(Branching logic for rectum operation)*

  ☐ R0
  ☐ R1
  ☐ R2

II. Surgical Approach

Approach

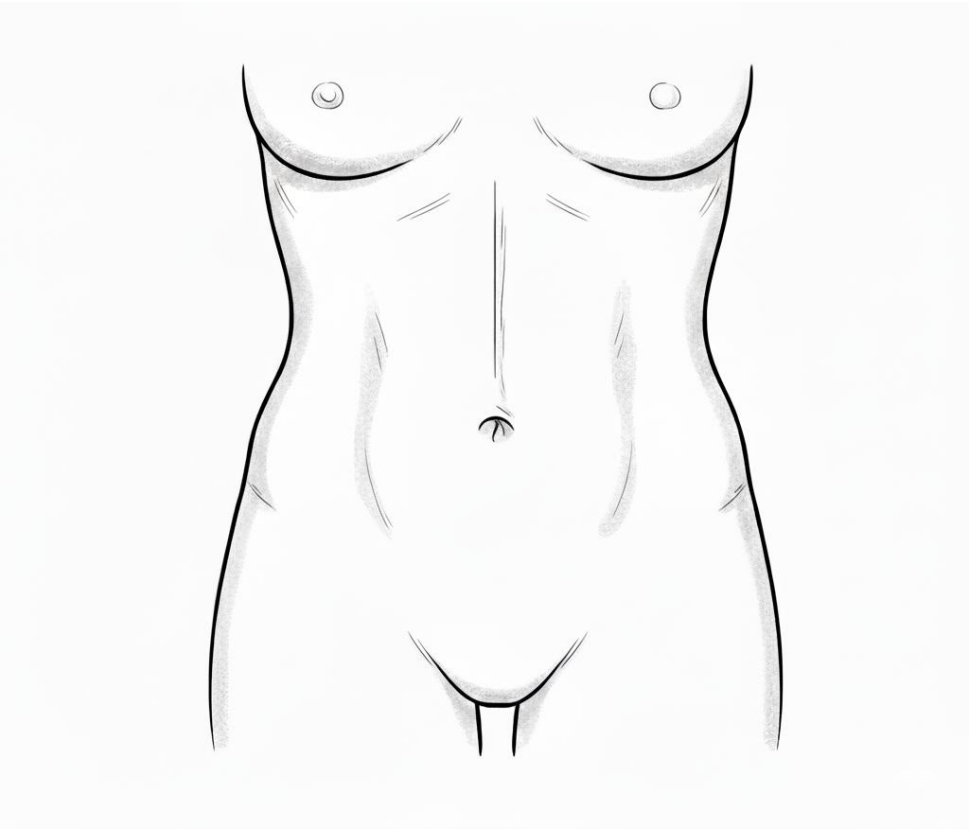
- **Surgical Approach:**
  - ☐ Open
  - ☐ Laparoscopic
  - ☐ Laparoscopic converted to open
  - ☐ Laparoscopic hand assisted
  - ☐ Transnal minimally invasive surgery (TAMIS)
  - ☐ Transanal endoscopic operation (TEO)
  - ☐ Transanal endoscopic microsurgery (TEM)
  - ☐ Robotic
  - ☐ Other

- **Reason for conversion (if applicable):** (*Branching logic from laparoscopic converted to open*)
  - ☐ Adhesions
  - ☐ Visceral injury
  - ☐ Vascular injury
  - ☐ Difficult exposure
  - ☐ difficult visualization
  - ☐ bleeding
  - ☐ failure to progress
  - ☐ Other: [Specify]

- **Access ports and incisions:**

  must be able to mark the following on the diagram:

**Ports and size -*	*5mm*
	10/11mm
	12mm
	15mm
Stoma site	*ileostomy*
	colostomy
Access incision	



III. Mobilization and Resection

Extent of Mobilization: [Check all that apply]

- ☐ Ascending colon
- ☐ Hepatic Flexure
- ☐ Splenic Flexure
- ☐ Descending colon
- ☐ Rectum
- ☐ Other

Vessel Ligation: [Check all that apply]

- ☐ Ileocolic
- ☐ Right colic
- ☐ Middle colic artery (Trunk)
- ☐ Right branch of Middle colic artery
- ☐ Left branch of Middle colic artery
- ☐ Left colic

- ☐ Inferior Mesenteric Artery (high ligation)
- ☐ Inferior Mesenteric Artery (low ligation)
- ☐ Other (Specify)

Inferior mesenteric vein ligation:

☐ None
☐ High
☐ low

Vessel Hemostasis technique: [Check all that apply]

- ☐ Ligaclips
- ☐ Hemoloc
- ☐ Diathermy
- ☐ Ultrasonic device (eg. Harmonic scalpel)
- ☐ Vessel Sealer (eg. Ligasure)
- ☐ Suture ligation
- ☐ Other (Specify)

- **Lymph Node Dissection (LND):**
  - ☐ D1
  - ☐ D2
  - ☐ D3
  - ☐ Other: [Specify]

- **Proximal transection:**
  - ☐ Ileum
  - ☐ Ascending colon
  - ☐ Transverse colon
  - ☐ Descending colon
  - ☐ Sigmoid colon
  - ☐ Proximal rectum
  - ☐ Mid rectum
  - ☐ Distal rectum

- **Distal transection:**
  - ☐ Ascending colon
  - ☐ Transverse colon
  - ☐ Descending colon
  - ☐ Sigmoid
  - ☐ Proximal rectum
  - ☐ Mid rectum
  - ☐ Distal rectum
  - ☐ Anal canal
  - ☐ Perineal resection

- **Anal Canal Transection level:** *(Branching logic from anal canal)*
  - ☐ Anorectal junction
  - ☐ Partial inter-sphincteric
  - ☐ Complete inter-sphincteric
  - ☐ Mucosal resection
  - ☐ Other: [Specify]

- **Excised *enbloc* resection:** [Check if removed with primary specimen]
  - ☐ None
  - ☐ Small bowel
  - ☐ large bowel
  - ☐ Bladder
  - ☐ Uterus
  - ☐ Abdomen wall
  - ☐ Other: [Specify]

IV. Reconstruction

Reconstruction: ☐ Anastomosis ☐ stoma ☐ other [specify]

- **Site of anastomosis:** *(Branching logic from anastomosis)*
- ☐ Intracorporeal ☐ Extracorporeal
- **Configuration:** *(Branching logic from anastomosis)*
  - ☐ End-end
  - ☐ End-side
  - ☐ Side-end
  - ☐ Side-side: Reverse peristalsis
  - ☐ Side-side: Isoperistaltic
  - ☐ Other [specify]
  -
- **Anastomotic technique:** *(Branching logic from anastomosis)*
  - ☐ Suture
  - ☐ Stapled
  - **Linear stapler sizes:** *(Branching logic from stapled)*
  - ☐ 45
  - ☐ 60
  - ☐ 75
  - ☐ 80
  - ☐ 100
  - ☐ Other
-
  - **Circular stapler sizes:** *(Branching logic from stapled)*
  - ☐ 27
  - ☐ 28
  - ☐ 29
  - ☐ 30
  - ☐ 31
  - ☐ 32
  - ☐ 33
  - ☐ Other
  -
- **Anastomotic height:** *(Branching logic from stapled)*
  - ☐ more than 5cm
  - ☐ less than 5cm

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    - **Suture material used:** *(Branching logic from suture)*
    - ☐ PDS
    - ☐ vicryl
    - ☐ Prolene
    - ☐ V-loc
    - ☐ Other [specify]

Anastomotic testing*(Branching logic for rectum operation)*

- **Doughnut assessment:** *(Branching logic from stapled)*

    ☐ Intact proximally and distally
    ☐ Proximal defect
    ☐ Distal defect

- **Air leak test**: *(Branching logic from stapled)*
    - ☐ No leak
    - ☐ Leak detected
    - ☐ Not done
- **Indocyanine Green (ICG) test:** *(Branching logic for rectum operation)*
    - ☐ done
    - ☐ not done

- **Stoma configuration (if applicable):** *(Branching logic from stoma)*
    - ☐ Loop ileostomy
    - ☐ End ileostomy
    - ☐ Loop colostomy
    - ☐ End colostomy
    - ☐ Other (Specify)

- **Reason for Stoma:** *(Branching logic from stoma)*
    - ☐ Surgical difficulty
    - ☐ High risk of anastomosis
    - ☐ Permanent stoma

- ☐ Diversion stoma
- ☐ Other: [Specify]

V. Operative Events & Closure

Points of Difficulty:

- ☐ None
- ☐ Adhesions
- ☐ Fibrosis
- ☐ Bleeding
- ☐ Tumour infiltration
- ☐ Anatomy exposure
- ☐ Bowel distension
- ☐ Limited operative space
- ☐ Equipment problems
- ☐ Anaesthetic problems
- ☐ Camera handling
- ☐ Assistant retraction
- ☐ Other: [Specify]

Intra-operative events/complications:

- ☐ None
- ☐ Bowel injury
- ☐ Vascular injury
- ☐ Adjacent organ injury: [Specify organ]
- ☐ Tumour perforation
- ☐ Bleeding: [Describe management]
- ☐ Stapler malfunction
- ☐ Anaesthetic events

- ☐ Other: [Specify]
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- **Specimen extraction site:**
  - ☐ Suprapubic ☐ Periumbilical ☐ Stoma site
  - ☐ Transanal ☐ Transvaginal ☐ Other: [Specify]

- **Wound protector used:** ☐ Yes ☐ No

- **Drain insertion:** ☐ Yes ☐ No

- **Type of drain:** *(Branching logic from drain site: Yes)*
  - ☐ Closed suction drain: [Specify location]
  - ☐ Closed Passive drain: [Specify location]
  - ☐ Open:
  - ☐ Other:

- **Intra-peritoneal drain placement:** *(Branching logic from drain site: Yes)*
  - ☐ Right subphrenic space
  - ☐ Right subhepatic space
  - ☐ Right paracolic gutter
  - ☐ Left subphrenic space
  - ☐ Left subhepatic space
  - ☐ Left paracolic gutter
  - ☐ Pelvis
  - ☐ Adjacent to anastomosis
  - ☐ Other: [Specify]

- **Exit site of drain**: *(Branching logic from drain site: Yes)*
  - ☐ Right upper quadrant
  - ☐ Right lower quadrant
  - ☐ Left upper quadrant

- ☐ left lower quadrant
- ☐ perineum
- ☐ Other: [Specify]

- **Fascial closure:**
  - ☐ 5mm port site
  - ☐ 10/11 port site
  - ☐ 12mm port site
  - ☐ 15mm port site
  - ☐ Access incision
  - ☐ Other: [Specify]

- **Fascial closure (Suture material):** *(Branching logic from any fascial closure tick)*
  - ☐ Vicryl ☐ PDS ☐ Ethibond ☐ Other: [Specify]

- **Skin Closure**
  - ☐ subcuticular interrupted
  - ☐ subcuticular continuous
  - ☐ Interrupted sutures
  - ☐ Continuous suture
  - ☐ Staples
  - ☐ Tissue glue
  - ☐ Adhesive strips
  - ☐ Other: [Specify]

- **Skin closure (Material/Method):** *(Branching logic from any skin closure tick)*
  - ☐ Monocryl ☐ V-Loc (V. Loc) ☐ Nylon
  - ☐ Staples ☐ Other

Additional information

Post operative management

Doctor

Signature

Date